

**CALIMESA FIRE DEPARTMENT
POLICY MANUAL
VOLUME 5
Policy: 501.02 EMS Apparatus Checkout**

11/2023

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Purpose

The purpose of this policy is to ensure accountability that all of the emergency medical supplies and equipment are at PAR levels, uncontaminated, functional and unexpired.

Policy

This policy applies to EMT-basics, AEMTs and EMT-Paramedics and it lists the required and optional equipment, along with corresponding minimum quantities, to be carried by each first response vehicle in Riverside County.

- EMT-Paramedics are responsible for thoroughly counting and examining all EMS equipment on each piece of apparatus they are assigned to and accountable for.
 - BLS medical bags (trauma, O2, C-spine & MCI)
 - ALS box including inner bags and containers (narcotics, IV, & IO)
 - Airway bag including inner bags (intubation)
 - Zoll monitor with self-check and defib check attached to EMS checkout
 - AED (if applicable)
 - Backup equipment will be checked out and kept in service.
- EMT-Paramedics at the beginning of their shift will complete narcotic checkout
- Any missing, damaged or expired medications will be replaced from the restock lockers and the amount taken will be noted on the CMS EMS Restock form.
 - All non-narcotic medications and equipment will be labeled out of service and placed in the out of service locker.
 - On-duty commanding officer and/or Battalion Chief will be notified of out of service equipment.
 - On-duty commanding officer and/or Battalion Chief will be notified if EMS supply PAR levels cannot be met.
 - EMS Coordinator will be notified by on-duty commanding officer.
- All supplies and medications that will expire within 120 days, will be rotated to the front of the ALS box or EMS bag.
- All supplies and medications near or under 90 days will be placed out of service and exchanged with AMR (notify Captain and EMS Coordinator)
- Completion of the inspection and restock will culminate with the CMS EMS medical supply checkout filled out.
- CMS EMS checkout's are inspected by the shift Fire Captain weekly and the EMS Coordinator monthly
- Failure to complete, falsify or not complete the CMS EMS checkout will result in disciplinary.

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Policy: 501.03 BLS and ALS Restock

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Purpose

The purpose of this policy is to ensure that personnel shall follow the expectations of restocking basic life support and advanced life support equipment and supplies when below Riverside Emergency Medical Services Agency and the City of Calimesa Fire Department PAR levels.

Policy

- Fire personnel will attempt to restock supplies that were used on scene from the AMR unit once the incident is completed.
- Restock of supplies will not interrupt patient care or delay treatment.
- Engine restock will only be used when multiple patients have exhausted resources on scene.
- Fire station restock will be deemed backup supplies.
- AMR shall not restock narcotics, equipment or any supply that may put them under PAR levels and placing them out of service.
- Reference policy 501.02 EMS Daily Equipment Checkout for out of service notifications.
- Reference policy 501.10 Controlled Substance for restock of a controlled substance.

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Policy: 501.04 Documentation

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Purpose

The purpose of this policy is to establish and ensure consistent, complete and accurate documentation of all emergency medical services (EMS) incidents responded to by the City of Calimesa Fire Department, while outlining information that is critical for each section.

The California Code of Regulations Title 22, Section 1001715 (A) (6) and Riverside Emergency Medical Services Agency (REMSA) protocol 7701 specifies the requirements for the initiation, completion, continuous quality improvement and retention of a patient care record. The ePCR/PCR is the permanent legal medical record that documents all aspects of prehospital care or refusal of care. The ePCR/PCR provides pertinent patient information to other healthcare providers regarding patient presentation and field care provided. The ePCR/PCR is subject to subpoena and may be used as evidence to litigation within the California court systems.

Policy

Patient Care Report (PCR)

- City of Calimesa will utilize the county approved electronic platform ImageTrend Elite to securely complete an electronic patient care report (ePCR)
- In the event that an ePCR cannot be utilized a multi-copy paper PCR can be used as an on scene substitute
- A patient care report (PCR) must be completed on every emergency medical response by an EMT-Paramedic, EMT-Basic and/or AEMT and in accordance with REMSA protocol 7701. Required fields:
 - Incident data
 - Patient information (Blackout or demographics)
 - Disposition
 - Treatment/Evaluation
 - Narrative
 - Signatures
- An ePCR/PCR must be initiated by each EMS provider, for every response, regardless of patient disposition.
 - When two (2) or more apparatus/units from the same EMS provider are dispatched, the EMS field personnel with the highest level of certification from each separate apparatus/unit are required to initiate and complete an ePCR/PCR.
 - Cancelled enroute or on scene will still require an ePCR/PCR
- The ePCR must accurately and completely document the patient's response, and care provided, while including all required information.
- Prehospital providers will only document assessments and procedures that were performed, and/or medications that were administered, by personnel within their own organization with one (1) exception: in the event that CPR is performed, or medications are administered by laypersons prior to first responder arrival, providers will document

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Policy: 501.01 EMS Compliance

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those procedures and/or medications in the applicable ePCR fields as those lay persons are not participants in the organized EMS system.

- In the case of an MCI / MPI incident, a minimum of one (1) ePCR must be completed by the first responder agency, with the required ICS form(s) or worksheet(s) attached within the ePCR.
- Against Medical Advice (AMA) require a signature from no less than the patient or power of attorney and a witness.
- Against Medical Advice (AMA) parties under police custody and the patient is unable to sign can be released to law enforcement custody after base hospital contact and the officers badge number and name obtained.
- All created ePCRs must be completed and uploaded to the ImageTrend server with a validation score of 100. Any record with a validation score of 98 or less will be considered incomplete and will be subject to review and reconciliation to address deficiencies. ePCRs from both the transport agency and the first response agency (if applicable) must be completed and posted to the ImageTrend server within two (2) hours of completion of the response.
- ePCR upload requirements may be delayed due to an emergency response; however, submission must be completed as soon as possible but no later than the end of the shift when the patient contact occurred.
- Within 24 hours of notification, EMT-paramedics will be required to complete missing and/or incomplete ePCRs, unless they are off duty. In those instances, paramedics will have until the end of their next scheduled shift to complete missing and/or incomplete ePCRs.
- Addendums must be done with the assistance of the EMS Coordinator.
- Any record contained in the Riverside EMS information system (REMSIS) version of Elite maybe made available to system participants (prehospital/hospital) for shared patients through established practices when that system participant has a clinical need related to direct patient care or if/when required by the Riverside County EMS System Quality Improvement program and associated policies.
- All Patient care records shall be securely retained for at least seven (7) years, or for two (2) years after the patient reaches the age of majority (18 years of age), whichever is longer. Privacy will be protected by compliance with the Health Insurance Portability and Accountability Act (HIPAA).
- Disciplinary action may be taken by City of Calimesa Fire Department, Riverside Emergency Medical Services Agency or Emergency Medical Services Agency for alteration or falsification of information on the ePCR/PCR.